Antidepressants drugs

Goal of these lecture

By the end of this lecture, all students will be able to acquire complete knowledge, skills and positive attitude and comprehension toward antidepressants drugs.

Objectives

- Define antidepressant drugs and explain their mechanisms of action.
- Identify the clinical indications for antidepressant drugs and justify their use in the management of depression and other psychiatric disorders.
- Discuss the nursing role in caring for patients receiving antidepressant therapy, including assessment, medication administration, monitoring for side effects, patient education, and psychosocial support.
- Analyze factors influencing the selection of antidepressant drugs based on individual patient needs and preferences.
- Evaluate the effectiveness of antidepressant therapy in improving patient symptoms and quality of life, and develop a comprehensive patient and family education plan.

Introduction

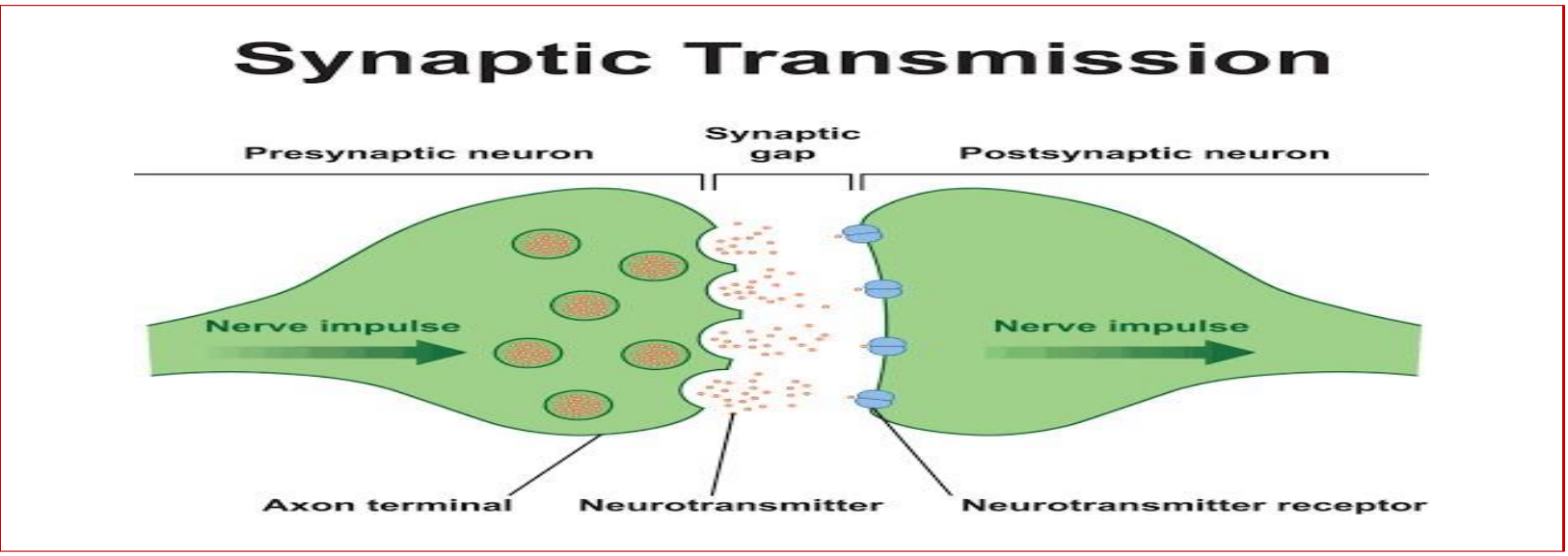
Antidepressants are medications that act on brain chemistry, especially neurotransmitters such as serotonin, norepinephrine, and dopamine. They are prescribed to improve mood and reduce symptoms of depression and related disorders. They work by altering the levels of certain chemicals in the brain, such as serotonin, norepinephrine, and dopamine, which can affect mood and emotions. There are several types of antidepressants available, including selective serotonin reuptake inhibitors (SSRIs), serotonin-norepinephrine reuptake inhibitors (SNRIs), tricyclic antidepressants (TCAs), and monoamine oxidase inhibitors (MAOIs). It is important to note that antidepressants should only be taken under the guidance of a healthcare professional, as they can have potential side effects and may interact with other medications.

Definition of Antidepressant drugs

An antidepressant is any drug that reduces depressive symptoms and helps regulate mood. While designed for depression, these medications are also useful in conditions like anxiety, insomnia, and chronic pain.

Indications of Antidepressant drugs:

They are mainly prescribed for major depressive disorder but are also effective in generalized anxiety disorder, panic disorder, obsessive–compulsive disorder, post-traumatic stress disorder, social phobia, and certain sleep and pain disorders



Onset of Action of Antidepressants:

- Antidepressant medications do not work immediately.
- Initial improvement in sleep, appetite, and energy may appear after 1–2 weeks.
- Mood improvement and reduction of depressive symptoms usually require 4–6 weeks, and sometimes up to 8 weeks.
- Early side effects may occur before therapeutic effects are noticed.
- Patients should be educated that continuing the medication as prescribed is essential**,** even if they do not feel better right away.
- Abrupt discontinuation should be avoided to prevent withdrawal symptoms and relapse.

Groups of antidepressant drugs

- Selective Serotonin Reuptake Inhibitors (SSRIs)
- Tricyclic Antidepressants (TCAs)
- Monoamine Oxidase Inhibitors (MAOIs)
- Serotonin-Norepinephrine Reuptake Inhibitors (SNRIs)
- Norepinephrine–Dopamine Reuptake Inhibitors (NDRIs)

Selective serotonin reuptake inhibitors

Pharmacodynamics of SSRIs

SSRIs selectively inhibit the reuptake of serotonin at the presynaptic neuronal membrane.
This leads to increased serotonin concentration in the synaptic cleft.
Enhanced serotonergic neurotransmission improves mood, anxiety, and other depressive symptoms.
They have minimal effect on norepinephrine and dopamine compared to other antidepressants

Pharmacokinetics of SSRIs

Absorption: Well absorbed from the gastrointestinal tract.
Distribution: Highly protein bound, widely distributed, cross blood-brain barrier.
Metabolism: Mainly metabolized in the liver.
Excretion: Excreted in urine and feces.

Side effects of SSRIs

Common Side Effects:

- Gastrointestinal: nausea, vomiting, diarrhea, dyspepsia
- Central nervous system: headache, insomnia or somnolence, dizziness, nervousness
- Sexual dysfunction: decreased libido, delayed ejaculation, anorgasmia
- Weight changes: mild weight gain or loss (varies by agent)
- Sweating and tremor

Less Common / Serious Side Effects:

- Gastrointestinal bleeding: risk increases with NSAIDs or anticoagulants
- Serotonin syndrome: if combined with other serotonergic drugs (e.g., MAOIs, triptans)
- QT interval prolongation: more notable with citalopram/escitalopram at higher doses
- Activation of mania or hypomania: in susceptible individuals (e.g., bipolar disorder)

Discontinuation (Withdrawal) Symptoms:
If abruptly stopped: flu-like symptoms, irritability, dizziness, insomnia, sensory disturbances.

Drug interactions of SSRIs
With drugs:
MAOIs and TCAs: lead to a potentially life threatening condition called serotonin syndrome: mental status changes, agitation, diarrhea, ataxia, myoclonus, shivering, tachycardia, increased reflexes, pyrexia, sweating.
NSAIDs and anticoagulants: increase the risk of bleeding, anticoagulant medications, such as warfarin.
Benzodiazepines and sedatives: enhance the sedative effects of benzodiazepines and other sedative medications, leading to increased drowsiness and impaired cognitive function.
With food:
Alcohol: Alcohol can worsen the side effects of SSRIs, such as drowsiness, dizziness, and impaired coordination.

Mnemonic to recall serotonin syndrome manifestations: "Madam's tips"
M = mental status changes A = agitation
D = diarrhea
A = ataxia (poor muscle control)
M = myoclonus(quick jerking movement that you can't control)
S = shivering
T = tachycardia
I = increased reflexes
P = pyrexia
S = sweating

Contraindications & Risks of SSRIs
MAOIs: All SSRIs are contraindicated in patients taking MAOIs, as the combination may cause serotonin syndrome.
CNS Depression: There are additive risks of central nervous system depression and psychomotor impairment when SSRIs are combined with alcohol, benzodiazepines, opioids, or other sedative drugs.
Bipolar Disorder: SSRIs are not recommended as monotherapy in bipolar depression because they may induce mania (exception: fluoxetine in combination with olanzapine).

LIST OF SSRIS

Generic name	Brand name	Dose
• Fluoxetine	Prozac	• 20mg/day
• Citlopram	Celexa	• 20mg/day
• Sertraline	Zoloft-Serlift	• 50mg/day

Tricyclic antidepressants TCAs

Border mechanism of action : block reuptake of serotonin and norepinephrine.

More effective than SSRIs in severe or treatment-resistant depression, but less well tolerated due to side effects.

Pharmacodynamics of TCAs:

Tricyclic antidepressants exert their effects by inhibiting the reuptake of the main neurotransmitters in the brain, serotonin (5-HT) and norepinephrine (NE), through their respective transporters (SERT and NET). This inhibition increases the concentration of these neurotransmitters in the synaptic cleft, enhancing neural communication and improving mood.

Pharmacokinetics of TCAs:

• **Absorption:** rapid oral absorption

• **Distribution:** Lipophilic property allows wide distribution, including the brain.

• **Metabolism:** Undergoes 1st-pass metabolism Metabolized in liver Excreted via kidney

• **Excretion (TCAs):** Mainly via kidneys as metabolites; minimal unchanged drug; minor fecal excretion.

Side effects of TCAs:

Central nervous system: Sedation, confusion (especially in elderly), lowered seizure threshold.

Anticholinergic effects: Dry mouth, blurred vision, constipation, urinary retention, tachycardia, cognitive impairment, glaucoma.

Antiadrenergic effects: Orthostatic hypotension, dizziness, erectile dysfunction (less common).

Cardiac effects: QT prolongation, arrhythmias (dangerous in overdose).

Metabolic/other: Weight gain, increased appetite.

Sexual dysfunction: Possible, but more common with SSRIs.

Long-term risks: ↑ Bone fractures (indirect factors: sedation, dizziness; some evidence of osteoporosis with prolonged use).

Psychiatric: Antidepressants may be associated with ↑ suicidal thinking, especially in young patients early in treatment.

Drug interactions of TCAs:

With drugs:

MAOIs and SSRIs: Serotonin syndrome

Anticholinergic drugs: TCAs have anticholinergic effects, and when combined with other anticholinergic medications like antihistamines, antipsychotics, or some urinary medications, they can increase the risk of side effects such as dry mouth, blurred vision, urinary retention, and constipation.

Cont. Drug interactions of TCAs:

CNS depressants: TCAs can enhance the sedative effects of other central **nervous** system (CNS) depressants like alcohol, benzodiazepines, and opioids. This can lead to increased drowsiness, dizziness, and impaired coordination.

TCAs may potentiate the cardiovascular effects of sympathomimetic agents (increasing the risk of hypertension and arrhythmias) and may also interfere with their therapeutic action.

With food:

Generally, no significant food interactions.

Patients should mainly avoid alcohol, as it increases CNS depression and sedation.

Contraindications of Tricyclic Antidepressants (TCAs)

1. Absolute Contraindications:

- Recent myocardial infarction: TCAs may exacerbate cardiac conduction abnormalities and increase the risk of arrhythmias.
- Known hypersensitivity to any TCA compound.
- Concurrent use of monoamine oxidase inhibitors (MAOIs): This combination may precipitate hypertensive crisis or serotonin syndrome.

2. Relative Contraindications / Conditions Requiring Caution:

- Cardiovascular disorders: including arrhythmias, conduction defects, or severe heart disease.
- Seizure disorders: TCAs can lower seizure threshold and increase seizure risk.
- Glaucoma: particularly narrow-angle glaucoma, due to anticholinergic effects.
- Urinary retention or prostatic hypertrophy: anticholinergic properties can worsen urinary symptoms.
- Hepatic impairment: since TCAs are extensively metabolized in the liver.
- Bipolar disorder: risk of inducing manic episodes.
- Pregnancy and lactation: TCAs should be used only if potential benefits

LIST OF TCAS

Generic name	Brand name	Dose
• Amitriptyline	Elavil-Saroten	• 25-150mg/day usually at bedtime
• Nortriptyline	Nortrilen-Avently	• 25-150mg /day usually at bedtime
• Imipramine	Tofranil-Janimine	• 75-15mg/day divided into 2-3 doses

Monoamine oxidase inhibitors MAOIs

Pharmacodynamics of MAOIs

Monoamine oxidase (MAO) enzyme widely distributed in neural tissue, liver, and the gastrointestinal tract. It plays a key role in the oxidative deamination of monoamine neurotransmitters.

MAO-A preferentially metabolizes serotonin, norepinephrine, epinephrine, and tyramine.

MAO-B preferentially metabolizes dopamine and tyramine.

Monoamine oxidase inhibitors (MAOIs) act by blocking the activity of MAO enzymes, either reversibly or irreversibly. This inhibition prevents the degradation of monoamines, resulting in increased intracellular stores of serotonin, norepinephrine, and dopamine. Consequently, larger amounts of these neurotransmitters are released into the synaptic cleft, enhancing neurotransmission and producing an antidepressant effect.

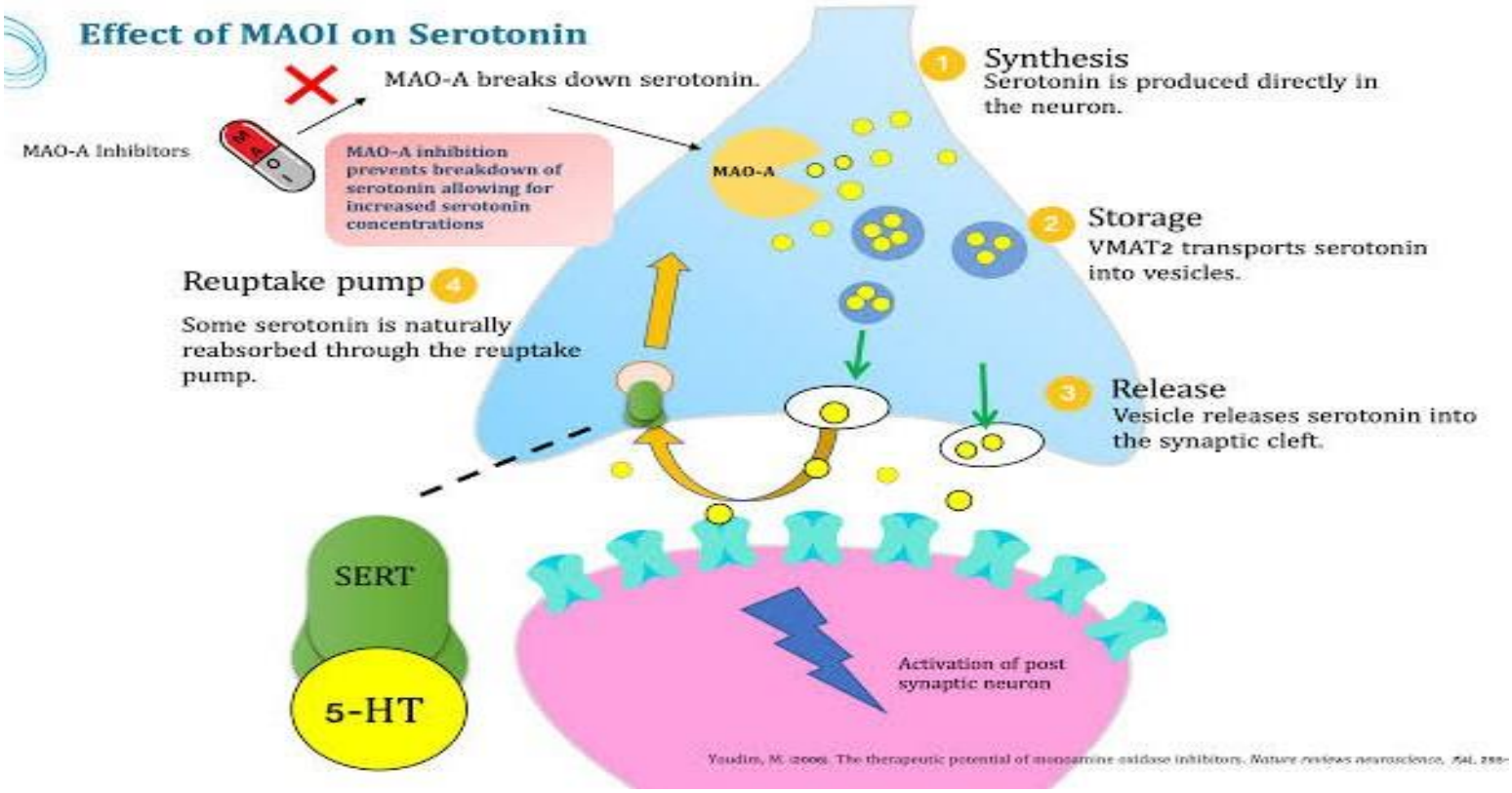
Pharmacokinetics of MAOIs

Absorption: Well absorbed from the gastrointestinal tract.

Distribution: Widely distributed, cross the blood-brain barrier.

Metabolism: Extensively metabolized in the liver.

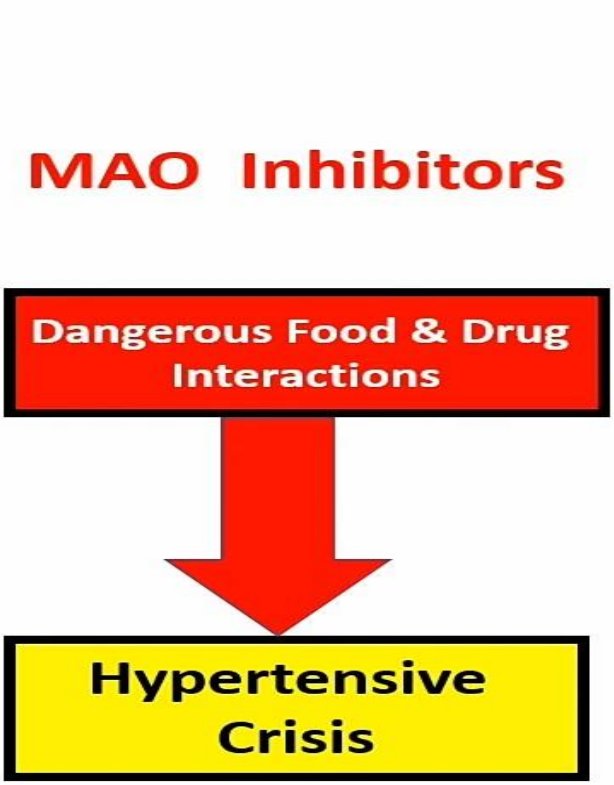
Excretion: Excreted mainly in the urine.

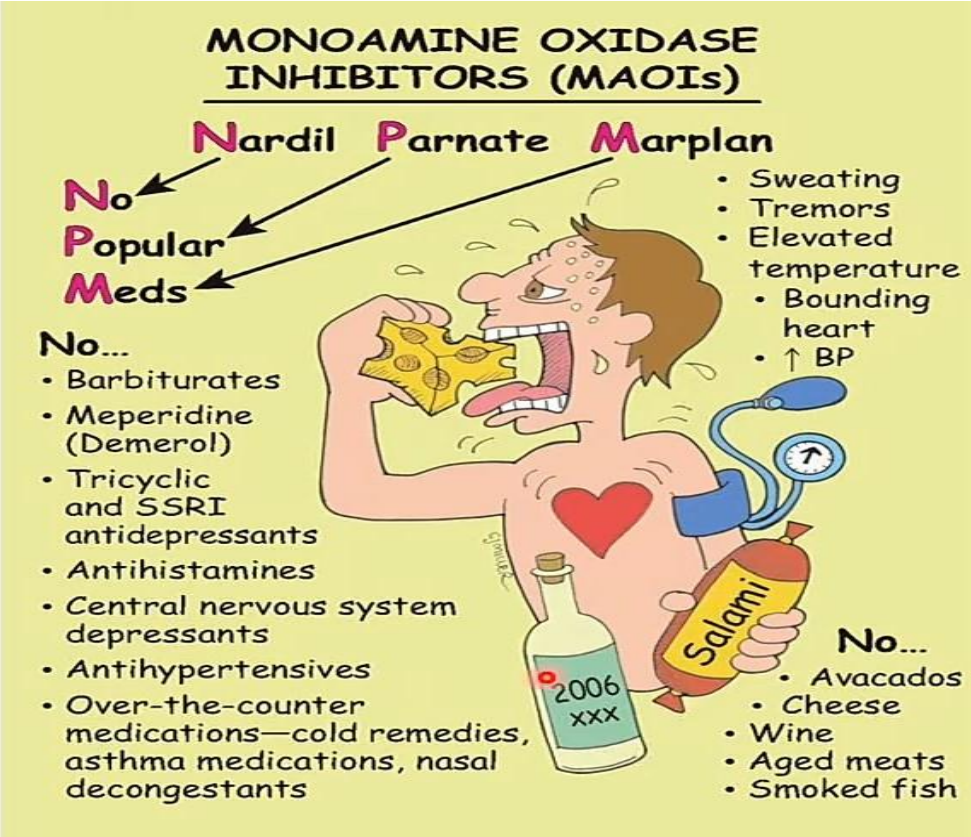


Drug interactions of MAOIs
Interaction with drugs

- **Serotonergic drugs**: Concomitant use with medications that increase serotonin levels in the central nervous system may result in serotonin syndrome, a serious condition that can present with symptoms such as hyperthermia, muscle rigidity, confusion, and autonomic instability.
- **Sympathomimetic drugs**: Concurrent administration may cause a hypertensive crisis due to excessive adrenergic stimulation.

  Note: Adequate washout periods are essential when switching to or from MAOIs to minimize the risk of dangerous interactions.
- With food:
- Foods rich in tyramine, such as aged or fermented products, smoked meats or fish, certain aged dairy products, and unpasteurized fermented beverages, can trigger severe increases in blood pressure when combined with MAOIs.
- This occurs because enzyme inhibition prevents the breakdown of tyramine, leading to its accumulation and the release of large amounts of norepinephrine, which in turn causes a hypertensive crisis.





Contraindications of MAOIs

- Cardiovascular disease (e.g., uncontrolled hypertension, heart failure, arrhythmias).
- Cerebrovascular disease (e.g., history of stroke).
- Liver impairment (reduced metabolism increases toxicity).
- Renal impairment (accumulation and higher risk of adverse effects).
- Thyroid disorders (risk of exaggerated response to sympathomimetic).
- Concurrent use of contraindicated drugs (e.g., serotonergic agents, sympathomimetic) due to risk of serotonin syndrome or hypertensive crisis.
- Pregnancy and lactation (safety not well established; potential risks to fetus/infant).

LIST OF MAOIS

Generic name	Brand name	Dose
• Phenelzine	Nardil	• 15-9mg/day
• Tranylcypromine	Parnate	• 10-60mg/day
• Isocarboxazid	Marplan	• 10-4mg/day

Serotonin and norepinephrine reuptake inhibitors. (SNRIs)

Pharmacodynamics:

They increase the levels of serotonin and norepinephrine in the brain by blocking their reuptake into nerve cells. SNRIs also have some effect on other neurotransmitters such as dopamine and acetylcholine, which may contribute to their therapeutic effects

Pharmacokinetics:

Absorption: well absorbed orally

Distribution: have a large volume of distribution

Metabolism: hepatic metabolism

Excretion: excreted mainly in the urine and feces.

Side effects of SNRI:
Common Side Effects

- Dry mouth, constipation or diarrhea, nausea
- Dizziness or lightheadedness, headache
- Decreased appetite or weight loss
- Sweating or hot flashes
- Sexual dysfunction

Serious / Rare Side Effects

- Changes in heart rate or blood pressure
- Seizures or convulsions
- Increased risk of bleeding (reduced platelet aggregation)
- Serotonin syndrome (especially with other serotonergic drugs)

Drug interactions of SNRIs
With drugs:
Other antidepressants: Such as MAO inhibitors, SSRIs or TCAs can also increase the risk of serotonin syndrome.
Blood thinners: SNRIs may interact with blood thinners such as warfarin and increase the risk of bleeding.
NSAIDs: like ibuprofen and aspirin may increase the risk of bleeding when taken with SNRIs.
Antipsychotics: Combining SNRIs with antipsychotic medications may increase the risk of side effects such as drowsiness, dizziness, and low blood pressure.
With food:
Alcohol: Can increase drowsiness, dizziness, and impaired judgment
Tyramine-rich foods: Not relevant for SNRIs (specific to MAOIs).

Contraindications of SNRIs
Concurrent or recent use of MAOIs (Monoamine Oxidase Inhibitors): Risk of serotonin syndrome or hypertensive crisis.

2. Relative Contraindications / Conditions Requiring Caution:

Uncontrolled hypertension: Some SNRIs (e.g., venlafaxine, duloxetine) may increase blood pressure.
Severe liver impairment: SNRIs are metabolized in the liver; dose adjustment or avoidance may be necessary.
Severe renal impairment: Dose adjustment may be required, especially for duloxetine.

Cont. 2. Relative Contraindications / Conditions Requiring Caution of SNRIs:

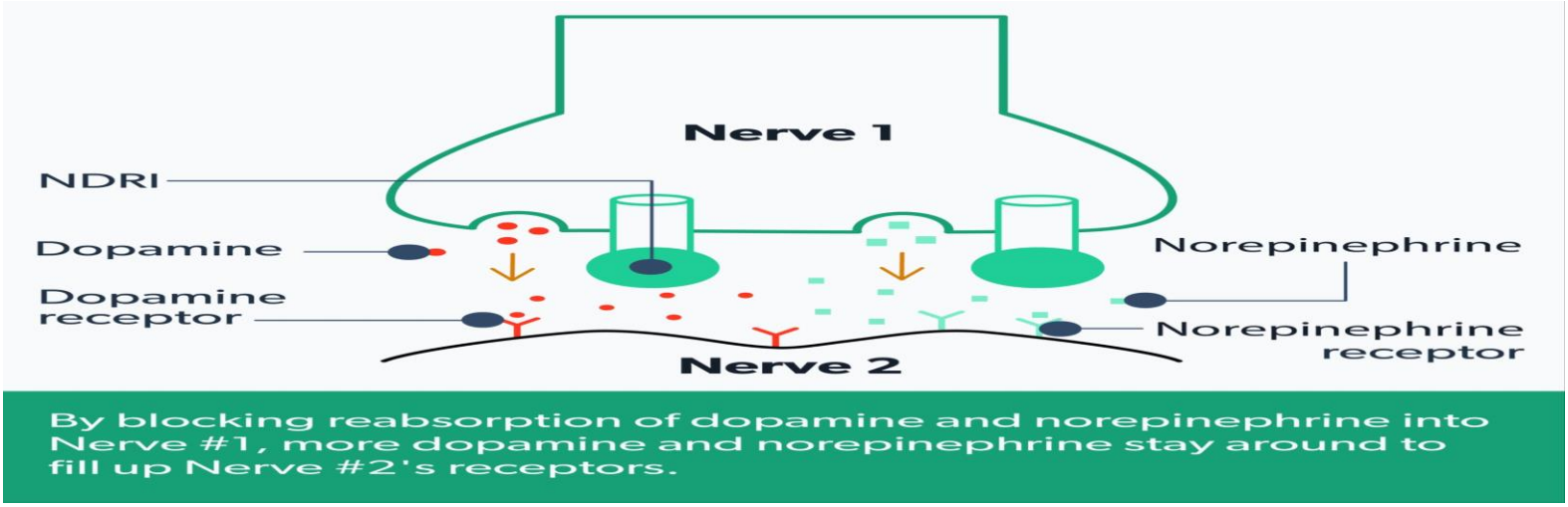
Bipolar disorder: Risk of inducing mania.
Pregnancy and lactation: Should be used only if benefits outweigh risks; some SNRIs are preferred over others.
History of seizures: Some SNRIs may slightly lower seizure threshold.History of seizures (some SNRIs may lower seizure threshold, especially at high doses)
Uncontrolled hypertension or severe cardiovascular disease (risk of elevated blood pressure or heart rate)
Pregnancy and lactation (use only if benefits outweigh risks)

LIST OF SNRIS

Generic name	Brand name	Dose
• Venlafaxine	Effexor	• 37.5-225mg/day
• Duloxetine	Cymbalta	• 20-60mg/day
• Desvenlafaxine	Pristiq	• 50-100mg/day

Norepinephrine dopamine reuptake inhibitor (NDRI)
Pharmacodynamics:
Inhibit norepinephrine (NE) and dopamine (DA) transporters (NET and DAT) in the presynaptic terminal → increase NE and DA levels in the synaptic cleft.
May indirectly enhance dopamine release, but the primary action is reuptake inhibition.

Pharmacokinetics of NDRI:
Absorption: Rapidly absorbed after oral administration; bioavailability may vary depending on the dosage form (immediate or extended-release).
Distribution: Widely distributed throughout body tissues; moderately bound to plasma proteins (approximately 80–85%).
Metabolism: Undergoes extensive first-pass hepatic metabolism, primarily via CYP enzymes (e.g., CYP2B6), producing active and inactive metabolites.
Excretion: Excreted mainly in urine as metabolites; a smaller fraction is eliminated in feces.



Side effects of NDRI

CNS: Insomnia, headache, agitation, tremor, dizziness (seizure risk mainly at high doses)

GI: Dry mouth, nausea, constipation

Cardiovascular: Mild tachycardia, possible hypertension

Metabolic: Weight loss (more common than gain)

Other: Increased sweating, occasional blurred vision

Drug interactions of NDRI

With drugs:

- Contraindicated with MAOIs (risk of hypertensive crisis).
- Bupropion inhibits CYP2D6, which can slow the breakdown of certain drugs (e.g., some SSRIs, antipsychotics, and beta-blockers), leading to higher blood levels and increased side effects.
- Use caution with stimulants (may increase risk of hypertension, tachycardia, and seizures).

With food:

No significant tyramine interaction; no special dietary restrictions needed (unlike MAOIs).

Avoid excessive caffeine or stimulants while taking NDRIs to reduce the risk of insomnia and anxiety.

Contraindications of NDRI

- **Concomitant use with MAOIs:**
- Do not administer bupropion with monoamine oxidase inhibitors; a washout period of at least 14 days is required to avoid severe hypertensive reactions.

Cont. contraindications of NDRI

- **History of seizures or epilepsy:**

Contraindicated in patients with epilepsy or a prior history of seizures, as bupropion lowers the seizure threshold.

- **Eating disorders:**

Contraindicated in patients with anorexia nervosa or bulimia nervosa due to an increased risk of seizures.

- **Angle-closure glaucoma risk:**

Not an absolute contraindication, but caution is advised in patients predisposed to increased intraocular pressure or angle-closure glaucoma.

List of NDRI:

Generic name	Trade name	Dose
Bupropion	Wellbutrin or Zyban	150mg-300mg per day
Atomoxetine	Strattera	20mg-100mg per day

Nursing Role in Caring for Patients Receiving Antidepressants Drugs

1. Assessment

- Assess patient's mental status, including mood, affect, thought content, and cognitive function.
- Obtain a comprehensive psychiatric history, focusing on depression, anxiety, sleep patterns, and presence of suicidal ideation or self-harm risk.
- Evaluate physical status: vital signs, weight, appetite, and sleep–wake cycle.
- Collect history of current and past medications, allergies, comorbid medical conditions, and substance or alcohol use.
- Assess baseline depressive symptoms, energy levels, and psychosocial functioning.
- Monitor adherence to treatment and evaluate the patient's knowledge and understanding of the prescribed antidepressant.

2. Medication Administration

- Administer antidepressants according to the prescribed dose, route, and schedule, ensuring adherence to treatment.
- Inform patients that therapeutic effects may take 2–4 weeks to become evident and reinforce the importance of continuous use.
- Monitor for immediate reactions or potential drug interactions.

3. Monitoring for Side Effects

- Observe for common side effects related to the specific class of antidepressant (e.g., gastrointestinal disturbances, sleep changes, sexual dysfunction, weight changes).
- Monitor for serious reactions such as serotonin syndrome (especially with combined serotonergic agents) or hypertensive crises (with MAOIs).
- Closely assess mood and suicidal ideation, particularly during initiation and dose adjustments.

4. Patient and Family Education

- Provide clear information on the purpose, expected benefits, onset of action, and potential side effects of the prescribed antidepressant.
- Emphasize adherence and the risks of abrupt discontinuation, especially with SSRIs and SNRIs, to prevent withdrawal symptoms.
- Educate on lifestyle measures that support recovery, including healthy sleep, nutrition, and exercise.

5. Collaboration with the Healthcare Team

- Work collaboratively with psychiatrists, psychologists, pharmacists, and social workers to develop and adjust treatment plans.
- Report any clinical changes or adverse effects promptly and ensure continuity of care and appropriate referrals.

6. Advocacy and Psychosocial Support

- Advocate for patient rights, including informed consent, confidentiality, and active participation in treatment decisions.
- Provide emotional support and help patients cope with stigma, build resilience, and maintain social and family support networks.

Conclusion:
The nursing role in antidepressant therapy is multifaceted and holistic. Through comprehensive assessment, safe medication management, vigilant monitoring, education, collaboration, and advocacy, nurses contribute significantly to improved treatment outcomes and enhanced patient quality of life.

Patient and Family Education for Antidepressant Therapy

- **Purpose of Antidepressants:** Educate patients and families that antidepressants are prescribed to manage depression and related mental health conditions. These medications help restore the balance of neurotransmitters in the brain, which can improve mood and emotional stability.

- **Types of Antidepressants:** Explain that there are several classes, including selective serotonin reuptake inhibitors (SSRIs), serotonin–norepinephrine reuptake inhibitors (SNRIs), tricyclic antidepressants (TCAs), monoamine oxidase inhibitors (MAOIs), and atypical agents. Each class differs in mechanism of action and potential side effects.

- **Expected Benefits:** Discuss the therapeutic effects, such as improved mood, reduction of depressive and anxiety symptoms, better sleep and appetite, increased energy, and enhanced overall quality of life.

- **Risks and Potential Side Effects:** Inform that common side effects may include nausea, dizziness, sedation or insomnia, sexual dysfunction, weight changes, and gastrointestinal disturbances. Emphasize that side effects vary between individuals and often lessen over time.

- Advise patients and families to promptly report any worsening of depression or emergence of suicidal thoughts, especially during the first few weeks or after dose adjustments.

- **Onset and Duration of Effect:** Explain that antidepressants typically require 2–6 weeks to achieve full therapeutic effect. Encourage adherence to the treatment plan during this period despite delayed improvement**.**

- **Medication Adherence:** Stress the importance of taking the medication exactly as prescribed. Abrupt discontinuation or missed doses can cause withdrawal symptoms and symptom relapse. Any changes in dosage should only occur under medical supervision.

- **Monitoring and Follow-Up:** Encourage regular appointments with healthcare providers for evaluation of therapeutic response, side effects, and necessary dose adjustments.

- **Lifestyle and Safety Considerations:** Educate about avoiding alcohol or non-prescribed medications unless approved by the provider. Highlight the importance of adequate sleep, balanced diet, and physical activity to support recovery.

- **Combination with Other Therapies:** Inform that optimal results are often achieved when pharmacotherapy is combined with psychotherapy, counseling, or other supportive interventions.

- **Open Communication and Support:** Encourage patients and families to express concerns and ask questions about the treatment. Emphasize the value of family and caregiver emotional support throughout the recovery process.

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Interactive Questions Based on the Lecture Content

1) True/False Questions

1. SSRIs mainly work by increasing serotonin levels in the synaptic cleft through inhibition of its reuptake.(….)
2. Antidepressants start working immediately within 24 hours of starting therapy. (….)
3. Common side effects of TCAs include dry mouth and constipation. (….)
4. Eating tyramine-rich foods while taking MAOIs can lead to a hypertensive crisis. (….)
5. NDRIs are contraindicated in patients with a history of eating disorders or seizures. (….)

2) Multiple Choice Questions (MCQs)

(Choose the correct answer)

1. Which of the following is an SSRI?
a) Fluoxetine
b) Amitriptyline
c) Venlafaxine
d) Phenelzine

2. Which of the following is a serious side effect when SSRIs are combined with other serotonergic drugs?
a) Constipation
b) Serotonin syndrome
c) Hypotension
d) Anemia

3. Which of the following is NOT an anticholinergic effect of TCAs?
a) Blurred vision
b) Dry mouth
c) Increased salivation
d) Urinary retention

4. Which of the following is an example of an MAOI?
a) Sertraline
b) Phenelzine
c) Bupropion
d) Duloxetine

5. Which of the following statements about the onset of antidepressant action is correct?
a) Mood improves immediately
b) Appetite and energy improve first within 1–2 weeks
c) Therapy should be stopped at the first sign of mild side effects
d) No medical follow-up is needed during therapy

3) Essay Questions

(Answer briefly and in an organized way)

1. Explain the mechanism of action of SSRIs and their effect on neurotransmitters?
2. List the main groups of antidepressants and give one example for each?
3. Describe the main signs and symptoms of serotonin syndrome and how to prevent it?
4. Discuss the nursing role in caring for patients receiving antidepressants (assessment – education – monitoring) ?
5. What dietary and drug precautions should be taught to a patient taking MAOIs?

"Here is a link to an educational video that you can watch to benefit from it."

https://youtu.be/4zaT-i3kT1g?si=CTZntsEcofWeOXT1